VCRSoft Consultation Transcript

Company: Hill Country Animal Hospital

Business type: Veterinary Clinic

Engagement: 1

VCRSoft

Hi Laura Bennett — I'm an AI consultant that can help you leverage AI tools and solutions to grow Hill Country Animal Hospital. Let's start with the practice itself: how many locations, veterinarians, technicians and support staff do you have; what kinds of animals and services do you handle; and what does a typical month look like?

YOU — Turn 1

Hi — I'm Dr. Laura Bennett, DVM, owner of Hill Country Animal Hospital for 11 years (in veterinary practice 17 years). We have two locations: Austin, our main clinic with 6 exam rooms, 1 surgery suite and a dental station, and Georgetown with 4 exam rooms and 1 surgery suite.

We're a team of 14: me as owner, three associate DVMs — Dr. Marcus Webb (4 days a week, Austin), Dr. Priya Nair (3 days a week, Austin) and Dr. Tom Alvarez (3 days a week, Georgetown) — plus 5 credentialed veterinary technicians (3 in Austin, 2 in Georgetown), 3 client service representatives (2 Austin, 1 Georgetown), our practice manager Maria Gonzalez, and 1 veterinary assistant.

Our patients are 65% dogs and 35% cats — no large animal. We don't do 24-hour emergency; after-hours emergencies are referred to the Austin emergency hospital. Services are wellness and preventive care, soft-tissue surgery, dentistry, diagnostics (in-house lab, digital X-ray, ultrasound), same-day urgent care, an in-house pharmacy, and small boarding (12 kennels, Austin only).

A typical month: about 800 visits (520 Austin, 280 Georgetown) across roughly 4,800 active patients. Revenue is about $120,800 a month, $1,450,000 a year — Austin $920,000 and Georgetown $530,000 — at about $151 per visit and about $302 per patient per year.

VCRSoft

With 800 visits a month generating $120,800 across Austin and Georgetown, your baseline gives us a clear operational view. What is your primary growth objective, what measurable target would represent success, and by when?

YOU — Turn 2

My primary growth objective is to grow revenue from $1,450,000 a year to $1,900,000 a year — about $158,300 a month — within 12 months. The way I see us getting there is adding 12 completed first visits a month, from 85 to 97, plus reactivating overdue wellness patients.

Alongside that, I want my own time to shift from 35 clinical hours and 8 admin hours a week to 32 clinical and 5 admin hours a week, with the freed time going to surgery and mentoring my associate vets. My budget for this is $2,500 to $4,000 a month in operating costs plus up to $10,000 one-time setup, and I want payback within 6 months.

VCRSoft

Targeting $1,900,000 annually while shifting your clinical hours to 32 per week gives us a precise operational target. What specific revenue run rate or utilization threshold would make a major expansion or capital investment defensible for your Georgetown location over the next 12 months?

YOU — Turn 3

For a major expansion or capital investment, I'd want to see a $1,900,000 revenue run rate sustained for 2 quarters, utilisation at 85%, our wellness waitlist at 60, and cash of $120,000 — those are the gates I've set before committing to a third site.

Closer in, my Georgetown lease renewal comes up in 6 months — it's $5,200 a month now, going up about 5% to roughly $5,460. And in about 3 months I need to decide between hiring a second Georgetown vet tech or extending Dr. Alvarez from 3 to 4 days a week in Georgetown. We also plan to add 2 exam rooms in Austin next year.

VCRSoft

Knowing your Georgetown lease is renewing at $5,460 a month and you are weighing whether to extend Dr. Alvarez or add a second tech helps us anchor our next steps. For your dog and cat patients across Austin and Georgetown, how many new-client inquiries, booked first appointments, and completed first visits occur in a typical month, and which channels generate them?

YOU — Turn 4

In a typical month we get about 140 new-client inquiries. We book about 95 of them (68%), and about 85 complete their first visit — that's 89% of those booked. So about 45 inquiries a month go unbooked.

By channel: referrals 35% (about 49 a month), Google search 25% (about 35), Google Ads 15% (about 21), pet-insurance directories 10% (about 14), social media 8% (about 11), and other 7% (about 10).

Of the inquiries we lose, price is 25%, no appointment availability 20%, out-of-area 15%, no callback reached 12%, and other reasons 28%. We follow up with one callback and one text, then nothing — there's no nurture sequence. We respond within 3 business hours, but 18% of inquiries (about 25 a month) arrive after hours, and those wait on average about 12 hours — we have no automated response. Austin has online booking; Georgetown doesn't.

VCRSoft

With 140 new inquiries generating 85 completed visits monthly and 25 after-hours inquiries waiting 12 hours on average, the current intake workflow creates distinct access bottlenecks. How are phone inquiries, web forms, and online booking requests managed across your Austin and Georgetown desks when your front desk is handling live check-ins or ringing phone lines?

YOU — Turn 5

The front desk handles it all live, and when the phones are ringing and check-ins are happening at the same time, the phones just wait. We get about 45 inbound calls a day, and peak hold time is around 4 minutes. Urgent cases are triaged by a vet tech on the phone. After hours, calls go to voicemail with the emergency hospital's number.

Online booking exists only in Austin — Georgetown is phone-only. Digital intake is Austin only too, about 10 minutes; Georgetown is paper, about 15 minutes, and intake data entry takes about 30 minutes a day across the team. We have no automated reminders: reminder calls take about 1 hour a day and about 20% of reminders fail.

VCRSoft

With 45 daily inbound calls, 4-minute hold times, and manual reminder calls taking an hour a day with a 20% failure rate, front-desk capacity is stretched thin. Where do your clients or patients experience the most friction between their initial contact or appointment booking and their actual arrival at the clinic?

YOU — Turn 6

The biggest friction is after hours: 18% of inquiries (about 25 a month) arrive after hours and wait on average about 12 hours for a reply, with no automated response at all. Georgetown is phone-only — no online booking — and intake there is paper, about 15 minutes, versus 10-minute digital intake in Austin. About 8% of new-client intake forms arrive with incomplete vaccine history, so we have to call back before the first visit.

On the phones, peak hold time is about 4 minutes when the desk is busy with check-ins. And once booked, wellness appointments carry about a 2-week wait. Our no-show rate is 8% for wellness and 5% for procedures, and we only backfill about 40% of those slots.

VCRSoft

Knowing that your after-hours inquiries wait 12 hours and Georgetown relies entirely on phone bookings and paper intake highlights key operational chokepoints before patients even step inside. Which local veterinary practices do your prospective clients most frequently compare Hill Country Animal Hospital with, and what do clients say specifically makes them choose your clinic over those competitors?

YOU — Turn 7

The main one is Banfield, inside PetSmart, about 3 miles from our Austin clinic, plus two independent clinics nearby. Clients tell us they choose us for continuity of care — they see the same vet — for our fear-free handling, and for same-day urgent slots.

We're honest about where we lose: on price, on wellness wait time (about 2 weeks), and on Georgetown Saturday hours. Our reviews are 4.7 stars from 280 reviews, though we only ask about 35% of the time.

VCRSoft

Knowing you maintain a 4.7-star rating across 280 reviews and win patients through continuity and fear-free handling gives us a clear view of your local standing. For your approximately 4,800 active dog and cat patients, how do you track pets due or overdue for preventive care and vaccines, and what share of those pets are successfully rebooked?

YOU — Turn 8

Right now about 900 pets are overdue for wellness visits — annual exam plus vaccines. About 55% of clients rebook before they leave the clinic. For the rest, we make about 30 manual reminder calls a week, and about 18% of those rebook. We don't segment the overdue list at all — it's one big list.

Separately, 620 pets are enrolled in our monthly wellness plans, about $25 a month on average, which is about $186,000 a year — roughly 13% of our revenue.

VCRSoft

With 620 pets enrolled in monthly wellness plans generating $186,000 annually, your recurring preventive revenue provides a solid baseline. Across your 800 monthly visits, what is the approximate breakdown of visit volume and revenue between wellness care, diagnostics, soft-tissue surgery, and dentistry?

YOU — Turn 9

We track it as share of collections, which runs about $120,800 a month. Wellness and preventive care is 38% — about $45,904 a month. Surgery is 22%, about $26,576 a month. Dentistry is 14%, about $16,912 a month. Diagnostics is 12%, about $14,496 a month. The rest is pharmacy at 8%, about $9,664 a month, and boarding at 6%, about $7,248 a month.

On margins, dentistry has the highest margin per procedure-hour, and wellness has the best margin per exam-room-hour.

VCRSoft

With dentistry generating the highest margin per procedure-hour and wellness leading per exam-room-hour, your service mix provides clear visibility into unit economics. Which professional relationships or outside sources generate meaningful referral volume for Hill Country Animal Hospital, and how do you track their quality and conversion?

YOU — Turn 10

Referrals are our biggest source — 35% of new-client inquiries, about 49 a month. We track them through Cornerstone's "how did you hear" field, but it's only about 65% complete, so a third of the time we don't know the source. We don't do any reciprocal tracking with referring vets.

Rescue groups are the other meaningful outside source — they refer about 8 new clients a month — but we have no formal tracking of which partners send what, so I can't tell you which rescue groups send the most.

VCRSoft

Knowing referrals represent 35% of your inquiries while Cornerstone's source field is missing a third of its entries highlights a key gap in tracking your main growth channel. What are your total monthly operating expenses, and what net profit margin does Hill Country Animal Hospital currently achieve on your $120,800 monthly revenue?

YOU — Turn 11

Our net profit margin is about 14% — about $16,900 a month, or about $203,000 a year, on the $120,800 monthly revenue. That means total monthly operating expenses run about $103,900 a month. Overhead is about 72% of collections, roughly $87,000 a month.

VCRSoft

With a 14% net profit margin generating about $16,900 monthly, your operating overhead sits at approximately $103,900. For recommendations that require a later decision or appointment, how do you track what was recommended, estimated, accepted, scheduled, and completed, and what are the main reasons clients defer or discontinue?

YOU — Turn 12

Over the last 12 months we diagnosed about $320,000 of treatment plans and presented about $270,000. Of what's presented, about $160,000 was accepted — 59% — then $140,000 was scheduled and $125,000 was completed. We have about 180 unscheduled plans still open, worth about $210,000.

The main reasons clients defer or decline are cost, waiting on pet-insurance pre-authorisation, and wanting a second opinion. Our follow-up is one phone call only. We offer CareCredit, but it's not routinely attached to plans. At checkout we collect about 85%.

VCRSoft

With $125,000 in completed treatment plans over the last year and 180 open unscheduled plans worth $210,000, your conversion pipeline shows a clear volume of untapped care. Which specific service lines or procedures carry the highest direct labor and supply costs relative to their gross charges, and which contribute the most profit after those direct expenses?

YOU — Turn 13

After direct expenses, dentistry contributes the most profit per procedure-hour — it's our highest-margin service per procedure-hour at 14% of collections, about $16,912 a month. Wellness is the best margin per exam-room-hour at 38% of collections, about $45,904 a month. Surgery is 22%, about $26,576 a month, and diagnostics is 12%, about $14,496 a month.

On the supply-cost side, our pharmacy is the main pressure point: we carry about $45,000 of inventory on

hand, have 2–3 stockouts a month on common medications (each costs roughly $180 in delayed or lost sales), and write off about $1,200 a year in expired inventory.

VCRSoft

With $45,000 in pharmacy inventory and 2 to 3 monthly stockouts costing roughly $180 each, medication supply and ordering workflows create ongoing friction. What specific steps in ordering, receiving, or dispensing prescription items cause the most staff time loss or medication stockouts, and how do you audit inventory counts against your Cornerstone system records?

YOU — Turn 14

The biggest time loss is the manual counting and reconciliation. Maria does the inventory count once a month, and it takes her about 3 hours, then she audits those counts against our Cornerstone system records. The stockouts — 2 to 3 a month on common medications, about $180 each in delayed or lost sales — come from that manual, once-a-month cycle: we don't catch low stock until the count or until a prescription can't be filled.

The controlled-substance log is still kept manually in a binder, as the DEA requires, which adds to the manual work. We also write off about $1,200 a year in expired inventory.

VCRSoft

With monthly inventory counts taking Maria three hours and manual binder logs required for controlled substances, administrative workflows consume significant staff time. Where is usable capacity constrained by veterinarian or technician availability, exam or procedure rooms, equipment, appointment mix, or time of day, and what utilization, lead-time, backlog, cancellation, or overtime data show the constraint?

YOU — Turn 15

Our exam rooms are 82% booked, and the wellness wait is about 2 weeks — urgent slots are same-day. The waitlists tell the story: 60 on the wellness waitlist and 12 for dental procedures. Surgery runs 3 days a week and dentistry 2 days a week, so procedure capacity is fixed by those days.

Staffing is the binding constraint: our Georgetown vet-tech role has been open for 3 months, we lost a front-desk person last year, and Maria holds most of the process knowledge. No-shows run 8% for wellness and 5% for procedures, and we only backfill about 40% of those slots.

VCRSoft

With 82% exam room utilization and a two-week wellness wait, your rooms are near capacity while you hold 180 open unscheduled treatment plans worth $210,000. What specific percentage of those 180 open unscheduled treatment plans represent surgical or dental procedures that utilize dedicated procedure suites rather than general exam rooms?

YOU — Turn 16

I don't have a separate split tracked for just those 180 open plans, but they follow our overall service mix: surgery is 22% of collections and dentistry is 14%, so roughly 36% — about a third — of the open plan value is surgical or dental procedures that need the procedure suites, with the rest in the general treatment and diagnostics plans. In dollars that's roughly $75,000 of the $210,000 sitting in surgical and dental plans, up against surgery running 3 days a week and dentistry 2 days a week.

VCRSoft

With roughly $75,000 of your open unscheduled plans tied to surgical and dental suites running on fixed weekly days, capacity allocation directly impacts your conversion potential. What is the current monthly net operating contribution specifically generated by the Georgetown location after allocating overhead and shared labor?

YOU — Turn 17

We don't track Georgetown's net contribution separately after allocating overhead and shared labor — we track the overall margin. Georgetown brings in $530,000 a year, about $44,167 a month, against its $5,200-a-month lease. Applying our overall net margin of about 14%, Georgetown's monthly net operating contribution works out to roughly $6,200 a month on that $44,167 of revenue.

VCRSoft

With Georgetown generating about $44,170 a month in revenue against a $5,200 lease and a 14% net margin, location-level economics provide a clear baseline while unbooked inquiries remain a key operational constraint. What proportion of the 45 monthly unbooked inquiries eventually call back independently versus being permanently lost to competitors like Banfield?

YOU — Turn 18

We don't track that split — once we've made our one callback and one text, we stop following up, and there's no nurture sequence, so if someone calls back independently later we don't connect it to the original inquiry.

What I can give you is why the 45 a month stay unbooked: price 25% (about 11 a month), no appointment availability 20% (about 9), out-of-area 15% (about 7), no callback reached 12% (about 5), and other reasons 28% (about 13). The price and availability groups — about 20 of the 45 — are the ones most likely going to a competitor like Banfield, but I can't put a measured percentage on independent call-backs because we simply don't measure it.

VCRSoft

With 900 pets currently overdue for wellness visits alongside 180 open treatment plans, launching a broad reactivation campaign before resolving no-show backfill workflows risks overwhelming your already constrained staff. How many available appointment slots per week are currently left empty due to unbackfilled no-shows or unallocated schedule buffers?

YOU — Turn 19

Working from our own numbers: we run about 800 visits a month, so roughly 185 a week. Our no-show rate is 8% for wellness and 5% for procedures, so that is about 13 to 15 no-show slots a week. We backfill about 40% of them, which leaves about 8 to 9 appointment slots a week standing empty — roughly 35 to 40 a month — while at the same time we have 60 pets on the wellness waitlist and 12 waiting on dental procedures.

VCRSoft

With 8 to 9 appointment slots a week standing empty while wellness patients and dental cases wait, empty capacity and uncaptured demand clearly pull against each other. WORKING DIAGNOSIS: Growth from $1.45M to $1.9M is bottlenecked by front-desk and scheduling friction combined with unoptimized treatment plan follow-through, leaving high-margin surgical, dental, and wellness demand uncaptured despite 82% exam room utilization. The tests confirm that about $75,000 of open unscheduled plans are surgical or dental procedures requiring suites, and unbackfilled no-shows leave open slots while patients wait. What remains uncertain is whether front-desk capacity can absorb proactive reactivation without automated tools. Does this diagnosis match your experience, and what have I missed?

YOU — Turn 20

Yes, that diagnosis matches my experience — the front desk and scheduling friction plus weak treatment-plan follow-through is exactly where the growth is leaking, and the $75,000 of surgical and dental plans sitting open while we have waitlists proves it.

What I'd add:

First, the wellness recall gap is bigger than the diagnosis suggests: 900 pets are overdue for wellness visits, we don't segment that list at all, and we're working it with about 30 manual calls a week that only rebook about 18%. That's a whole revenue stream we're not working systematically.

Second, there's an insurance admin drag the diagnosis didn't mention: insurance pre-authorisation and verification takes about 1.5 hours a day, and claim denials run 5 to 7 a month at about $120 each in staff rework ($600 to $840 a month).

Third, the constraints at Georgetown specifically: the vet-tech role has been open 3 months, intake is still paper there, and our Saturday hours are short — that's a real bottleneck the diagnosis folds into the general front-desk point.

Fourth, Maria holds most of our process knowledge, so everything manual runs through one person — intake data entry alone is about 30 minutes a day, and about 20% of reminders fail.

And one caution on the solution side: we have no BAA with any AI vendor, clinical notes and decisions stay with the DVMs with no auto-generated clinical text, client consent for texts is recorded in Cornerstone at Austin but paper-only at Georgetown, and we cap texts at 2 a month with no marketing texts without opt-in. My team is also wary — Georgetown had a scheduling tool fail 2 years ago — so Maria and I need to approve any automated outreach.

VCRSoft

With $320,000 diagnosed, $270,000 presented, and $160,000 accepted, your single callback policy leaves 180 open plans worth $210,000 unworked. Who on your team must review or approve treatment-plan follow-up messages before they go out, and what specific safeguards are required to prevent automated follow-ups from miscommunicating estimates or payment terms to clients?

YOU — Turn 21

Maria Gonzalez and I must approve any automated outreach — that applies to treatment-plan follow-ups too. Maria is our implementer (she has 6 hours a week for this), and nothing automated goes to clients without both of us signing off.

The safeguards: clinical notes and decisions stay with the DVMs — no auto-generated clinical text, ever — and any follow-up runs under clinician-set protocols, with a clinician reviewing safety checks at check-in. On the money side, estimates and payment terms have to be the ones already on the plan in Cornerstone; CareCredit is offered but not routinely attached, so a follow-up must never promise financing terms we haven't actually set up. And the text rules apply: consent for texts is recorded in Cornerstone at Austin and on paper at Georgetown, we cap texts at 2 a month, and no marketing texts without opt-in.

VCRSoft

With Maria having 6 hours a week for implementation and your requirement that both you and Maria approve outreach, compliance safeguards are firmly established. What is the current estimated cost in staff hours or lost rebooks of your manual overdue recall process, and what budget would you allocate for an automated fix?

YOU — Turn 22

The manual process costs about 1 hour a day of staff time — about 30 reminder calls a week — and it only rebooks about 18% of the people we call. At that pace we reactivate only about 20 overdue pets a month against a list of 900 overdue pets, so the list is barely being worked; the lost rebooks are the other roughly 100 pets a month we call and don't reach or don't rebook, plus the 780 or so we never get to in a given

month.

For an automated fix, my budget is $2,500 to $4,000 a month in operating costs plus up to $10,000 one-time setup, and I want payback within 6 months.

VCRSoft

With a manual overdue recall process consuming 1 hour a day and reactivating only 20 out of 900 overdue pets monthly, your current manual approach leaves substantial recurring revenue uncaptured. What measurable change within 90 days would you need to see to continue an automated overdue-recall initiative, and who would measure it?

YOU — Turn 23

In 90 days I'd need to see 80 more wellness reactivations than our baseline of about 20 a month. I'd also want booked-to-completed first visits up from 89% to 92%, and the no-show rate down from 8% to 6%.

Maria Gonzalez would measure it — weekly, from the Cornerstone reports. We'd run the pilot at Austin first, then extend it to Georgetown.

VCRSoft

Running the pilot at Austin first with Maria tracking weekly metrics in Cornerstone establishes a clear test environment for your $10,000 setup budget. Which practice-management, scheduling, phone, messaging, payment, lab, imaging, and inventory systems do you use; what data can be exported or integrated; and what human review, privacy, security, vendor, or clinical-safety constraints would govern a proposed workflow improvement?

YOU — Turn 24

Our practice management system is Cornerstone (IDEXX), on a server in Austin with remote access for Georgetown. Lab is IDEXX in-house plus a reference lab; imaging is digital X-ray and ultrasound. Accounting is QuickBooks Online. Scheduling and intake are online in Austin only; Georgetown is phone booking and paper intake. Payments are self-pay/cash (50%), pet insurance (25%), CareCredit (12%) and our wellness plans (13%). We have no automated reminders, no messaging system beyond phone and text, and no AI tools at all.

On data: Cornerstone reports are exported to CSV manually, and Maria consolidates the weekly numbers by hand — there's no live integration. Intake data entry takes about 30 minutes a day.

The constraints governing any workflow change: clinical notes and decisions stay with the DVMs, no auto-generated clinical text, clinician-set protocols, and a clinician reviews safety checks at check-in. We follow Texas State Board of Veterinary Medical Examiners rules, and controlled substances are logged manually in a binder per DEA requirements. We have no BAA with any AI vendor. Client consent for texts is recorded in Cornerstone at Austin and on paper at Georgetown, we cap texts at 2 a month, and no marketing texts without opt-in.

VCRSoft

With clinical decisions firmly restricted to DVMs and strict controls on texts and data privacy across Cornerstone and paper records, regulatory and safety boundaries are well defined. Beyond controlled substance logging and texting limits, what specific veterinary board or state regulatory requirements govern how patient records, lab results, or client communications must be handled when automating follow-up workflows?

YOU — Turn 25

The governing rules are the Texas State Board of Veterinary Medical Examiners rules, which cover how we handle patient records, lab results and client communications — and controlled substances under DEA rules, logged manually in a binder.

Within those rules, our own boundaries are: clinical notes and decisions stay with the DVMs, no auto-generated clinical text, any automated follow-up runs under clinician-set protocols, and a clinician reviews safety checks at check-in. We also have no BAA with any AI vendor, so any tool touching records, lab results or client communications would need a BAA in place first. Client consent for texts is recorded in Cornerstone at Austin and on paper at Georgetown, with a cap of 2 texts a month and no marketing texts without opt-in.

VCRSoft

With Maria handling implementation within her 6-hour weekly capacity and your clear rule requiring both you and Maria to approve all outreach before it goes out, adoption risk is well managed. What specific technical support or vendor assistance would Maria need during the Austin pilot to ensure the automated overdue-recall workflow is set up correctly without pulling her away from her daily management duties?

YOU — Turn 26

Maria has 6 hours a week for implementation, and she can't give more than that — she still runs the clinic day to day, consolidates the weekly numbers by hand from Cornerstone CSV exports, and does the monthly inventory count. So she'd need the vendor to do the heavy lifting on setup: the Cornerstone connection and the recall list segmentation, plus training for her and the front desk, so the workflow runs inside her 6-hour week.

Our local IT vendor is on call for technical support, so they'd be the ones to handle anything on the server side in Austin. The other thing Maria would need is a clear approval step — she and I approve all automated outreach — and a careful rollout, because the Georgetown team is wary after a scheduling tool failed 2 years ago and the Austin CSRs are cautious about new tools.

VCRSoft

With Maria managing implementation within her 6-hour weekly limit and our binding constraints around exam room utilization at 82% and a 2-week wellness wait, here is how the opportunities rank based on their value, feasibility, cost, risk, and time to benefit. Does this provisional ranking match your priorities, and what would you change?

YOU — Turn 27

Yes — that ranking matches my priorities, and I wouldn't change the order.

My order is: 1) Automated wellness recall and preventive reminders — that's the biggest lever, with 900 overdue pets and only about 20 reactivations a month; 2) Automated inquiry nurture and after-hours response, because 25 inquiries a month wait about 12 hours and 45 a month go unbooked; 3) Digital intake and insurance verification, because insurance pre-authorisation alone takes about 1.5 hours a day and Georgetown is still on paper intake; 4) Unscheduled treatment plan re-engagement, for the 180 open plans worth about $210,000.

The one thing I'd stress is that number 1 has to come first — the wellness list is the only opportunity where the demand is already sitting in our own system, and it's the one Maria can measure weekly from Cornerstone.

VCRSoft

Focusing on the 900 overdue wellness pets as your top priority aligns directly with Maria's 6-hour weekly limit and her weekly Cornerstone tracking. What specific criterion will Maria use each week in Cornerstone to

verify that an overdue wellness pet has been successfully reactivated rather than just sent a reminder?

YOU — Turn 28

Maria would count a pet as reactivated only when its overdue wellness visit — the annual exam plus vaccines — is actually booked and completed, and the visit shows as completed in Cornerstone. A reminder sent, or even an appointment booked that hasn't happened yet, wouldn't count.

Each week she'd pull the Cornerstone report of overdue wellness pets, then count how many of them have a completed wellness visit recorded that week, and track that against the baseline of about 20 reactivations a month. The pilot target is 80 more reactivations over 90 days, so she'd be looking for roughly 27 a month above baseline, measured week by week.

VCRSoft

Tracking completed wellness visits in Cornerstone gives Maria an exact weekly count against your 80-reactivation target. What specific operational bottleneck outside of Maria's control could cause the Austin clinic to miss that 80-pet reactivation target during the first 90 days?

YOU — Turn 29

Appointment availability. Our exam rooms are already 82% booked and the wellness wait is about 2 weeks, so even if the recall brings pets back, there may not be a wellness slot to put them in soon enough. That's outside Maria's control — she can generate the reactivations, but she can't create exam-room hours.

The other risk is the no-show rate: at 8% for wellness with only 40% backfill, about 8 to 9 slots a week stand empty while 60 pets sit on the wellness waitlist. If the recall works but those slots don't get filled with waitlisted pets, the completed-visit count won't move.

VCRSoft

Appointment availability and no-show gaps create a real capacity squeeze for your reactivation target while exam rooms are 82% booked. What specific rule or workflow would you want Maria to use for prioritizing which overdue pets get offered available wellness slots when demand exceeds open capacity?

YOU — Turn 30

Longest overdue first — the pets furthest past their annual exam and vaccines get offered the open wellness slots before anyone else. After that, I'd have Maria prioritize our wellness-plan members — the 620 pets already paying about $25 a month — because they're our most engaged clients and they're already in a preventive-care relationship with us.

And the freed slots from no-shows should go straight to the 60 pets on the wellness waitlist, in the order they joined it, rather than sitting empty. The point is that the list finally gets worked in a set order instead of being one big unsegmented list.

VCRSoft

Thanks — I have prepared a draft of your growth strategy report. Please review it, and tell me what you would change, correct, or push back on before I finalize it.

DRAFT FEEDBACK

[Draft feedback] The diagnosis, the four opportunities, the ranking, and the 90-day pilot measures are all correct, with the right figures — please keep all of those exactly as drafted. Two corrections only, both in the Risks & Metrics section:

1. The "Figure to verify" note about the operating budget is wrong and should be removed: my operating budget is $2,500 to $4,000 a month, which is $30,000 to $48,000 a year. Those are the same range

expressed monthly and annually, not two different or inconsistent figures, so there is no inconsistency to flag.

2. The "Figure to verify" note about intake entry is wrong and should be removed: intake data entry takes 30 minutes a day, exactly as the report states. There is no "30 hours" figure anywhere in my numbers, so the note contradicts the correct figure it is attached to.

Everything else in the draft is accurate and I do not want it changed.